INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Panitumumab

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
14 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00225
Version 6

> Panitumumab on Day 1 every 14 days while beneficial and tolerable. There is no home chemotherapy pump.

Treatment stages

01 **A repeating 14-day cycle**
Infusion → home → review

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

> Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Panitumumab
National Cancer Control Programme (NCCP) 00225

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathing difficulty, eye pain or blurred vision, blistering or infected skin, severe diarrhoea or dehydration promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How your treatment works

The aim is to control advanced bowel cancer and symptoms. The team reviews how well treatment is working and whether side effects remain acceptable; there is no single fixed number of cycles for everyone.

EGFR-TARGETED ANTIBODY

Panitumumab

Vectibix



Blocks growth messages at the cell surface

Panitumumab attaches to EGFR, a receptor on the outside of cells. This can interrupt signals that help some bowel cancers grow. A tumour RAS test helps identify whether this approach is suitable. EGFR also supports healthy skin, which explains why skin and nail changes need attention.

EGFR receives a growth signal → Panitumumab blocks the receptor → Some growth signals are reduced

What to watch for

Report rash, painful cracks or nail changes early. Eye pain or blurred vision, new breathing symptoms, severe diarrhoea, or blistering skin need prompt assessment. Blood tests check minerals such as magnesium.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim and review plan

Understand why this option fits your cancer.

The setting matters

Panitumumab is used for bowel cancer that has spread and has a confirmed wild-type RAS result. It is given after earlier fluoropyrimidine, oxaliplatin and irinotecan treatment has failed. Your team explains the aim, alternatives and how benefit will be reviewed.

What the tumour tests mean

RAS wild type is required for panitumumab. Your team also considers the cancer location, other molecular results, previous treatment and whether surgery or other local treatment could be useful. A positive eligibility result is not a promise of benefit.

20100007: panitumumab monotherapy

Studies can help explain why a treatment is offered. The schedule, previous medicines and tumour tests affect how well the evidence fits you.

Panitumumab improved survival in a trial group selected using tumour testing after other bowel-cancer medicines had stopped working. The likely benefit for you depends on your cancer, previous treatment and general health; it cannot be predicted from the trial alone.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment, review and the next cycle

Panitumumab

14-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

INFUSION → HOME → REVIEW

A repeating 14-day cycle

D1 D14

Day 1: panitumumab infusion. There is no home chemotherapy pump in this regimen. A cycle lasts 14 days. Treatment continues while it is helping and side effects remain acceptable.

A repeating reference cycle sits within a longer treatment plan. The oncology team chooses the number of cycles and confirms changes; no patient dates are required here.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

The columns show order, not equal periods. There is no fixed number of cycles for these plans. Agree who books each check, when results are reviewed and whom to contact if a check is missed.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00225 v6 · official regimen · Panitumumab product information

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Panitumumab: treatment and checks

Advanced / metastatic colorectal cancer

Panitumumab on Day 1 every 14 days while beneficial and tolerable. There is no home chemotherapy pump.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Repeated treatment	Longer-term review	Pause / completion
Blood counts, kidney and liver NCCP 00225	Baseline blood count, kidney and liver profile	Review results before treatment	Blood count, kidney and liver tests before each treatment	Repeat sooner if symptoms or results need it	Follow any unresolved abnormal result
Magnesium, calcium and potassium NCCP 00225	Check mineral levels before starting	Repeat periodically during treatment	Keep blood-test and replacement appointments	More checks may be needed if levels fall or diarrhoea develops	Monthly checks for two months after the last antibody dose
Skin, nails, eyes and breathing NCCP 00225	Discuss existing skin, eye or lung problems	Start the prescribed skin-care plan; report reactions early	Review skin, nails, diarrhoea and new symptoms	Eye pain, worsening breathing or severe rash needs prompt assessment	Continue care until symptoms and mineral levels settle
Cancer response and treatment choices NCCP 00225	Record baseline disease and agree review plan	Bring forward review if symptoms worsen	Scans and other tests follow your consultant's treatment plan	Discuss benefit, side effects and whether the goal has changed	Agree continuation, a break, another treatment or supportive care

What each check is for

1 A repeating 14-day cycle

Infusion → home → review

The aim is to control advanced bowel cancer and symptoms. The team reviews how well treatment is working and whether side effects remain acceptable; there is no single fixed number of cycles for everyone.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Visits, access and care between treatments

STEP 1

Before the first visit

Confirm the exact regimen, tests, medicines, emergency number and treatment goal. Ask whether a peripheral cannula or an existing line is appropriate; a port is not automatically required.

STEP 2

At the treatment unit

The first panitumumab infusion usually takes about one hour; higher doses or reactions may need longer. Later tolerated infusions may be shorter. Blood review, preparation, premedicines and observation add time; infusion time is not the total appointment length.

STEP 3

After the infusion

There is no fluorouracil home pump and no disconnection visit in this regimen.

STEP 4

Between doses

Use the written symptom and support plan; do not wait for the next appointment if urgent symptoms appear.

STEP 5

The next decision

Confirm the next visit after any delay or dose change. Treatment continues only while the oncology team judges benefit and tolerability acceptable. Follow-up mineral checks continue after the final EGFR-antibody dose where applicable.

Transport and daily life

Plan transport if antihistamines, symptoms or other medicines affect driving. Ask for practical, work, nutrition or emotional support.

If treatment changes

Ask which component has been held and which visits remain. A treatment delay does not automatically cancel blood tests, line care or symptom follow-up.

Before you start and before each cycle

The team checks that the plan remains suitable.

Checks and medicines

Review blood counts, kidney/liver function and minerals, current symptoms, allergies and all medicines. dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) testing is not a requirement for panitumumab alone.

Skin, eyes and breathing

Tell the team about an existing rash, skin infection, dry or painful eyes, contact lenses, or previous lung inflammation. Report new eye pain, light sensitivity or visual change promptly.

When treatment changes or finishes

Follow-up continues after the last infusion.

Monitoring and recovery

Continue your skin plan and planned magnesium, calcium and potassium checks for about eight weeks after panitumumab. New symptoms still need review. Ask when to stop prescribed supportive medicines.

Treatment choices

Your team may pause, reduce, change or stop one or more medicines. Ask which medicine changes, why, and what the next review will assess. Supportive and palliative care can help alongside active treatment.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Panitumumab infusion	 **Day 1** intravenous (IV)
Skin / nails	 **Days 1–14** May develop or accumulate over cycles
Bowel / fatigue	 **Days 1–14** Timing varies
Mineral changes	 **Days 1–14** Blood tests during and after treatment

The planned treatment: Day 1: panitumumab infusion. There is no home chemotherapy pump in this regimen. A cycle lasts 14 days. Treatment continues while it is helping and side effects remain acceptable.

Blood tests after the final dose: Mineral checks continue for about eight weeks after panitumumab. There is no guaranteed safe day for symptoms.

Skin care continues between visits: Rash or nail changes may build over several cycles. Report symptoms early and follow your written plan.

Why these bands stay broad: Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00225 v6 · official regimen ↗ · Panitumumab product information ↗

There are no guaranteed "safe days". Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Between-treatment notes

Day 1: Review and infusion

The team checks symptoms, blood tests and treatment suitability, then gives panitumumab through a vein. Tell the nurse immediately about infusion symptoms. Confirm the skin-care plan, contact number and next visit.

Day 2–7: At home

There is no chemotherapy pump to disconnect. Watch skin, bowel and eye symptoms and use the prescribed care plan. Report changes early; serious symptoms can happen on any day.

Day 8–14: Continue care and prepare for review

Rash, nail changes and mineral problems may build over several cycles. Feeling well does not replace blood tests. Bring your symptom record. Do not wait for the next infusion if symptoms are concerning.

Support around the medicines

1 **Start skin care early**

Ask for your skin-care plan before the first infusion. Use the recommended moisturiser and sun protection; your team may prescribe a cream or an antibiotic. Do not use ordinary acne treatments without advice.

2 **Minerals and blood tests**

Panitumumab can lower magnesium and other minerals even when you feel well. Blood tests continue during treatment and for about eight weeks after it finishes. Take replacement treatment only as prescribed.

3 **Feeling sick or loose stools**

Panitumumab has low nausea risk, but symptoms can still occur. Follow the rescue medicine and hydration plan from your team. Severe or persistent diarrhoea, reduced urine or inability to drink needs urgent advice.

4 **Travel and daily life**

Allow time for blood tests, review, preparation, infusion and observation. Ask about transport if medicines or symptoms affect driving. Pace activity and seek help with food, work or caring responsibilities.

5 **The whole treatment plan**

Ask how and when the team will assess benefit, which blood tests continue during a pause, and what happens after the final dose. Mineral checks continue monthly for two months after the last antibody treatment.

Living with treatment

Build the plan around what matters to you.

Skin and nails

Ask for your skin-care plan before the first infusion. Use the recommended moisturiser and sun protection; your team may prescribe a cream or an antibiotic. Do not use ordinary acne treatments without advice. Avoid friction on sore skin and tell the team about painful nail folds.

Food, activity and support

Keep fluids and nourishing food within reach. Pace activity around energy levels and ask for dietetic, emotional or practical support. If symptoms prevent eating, drinking or usual tasks, contact the team.

Pregnancy and fertility

Discuss pregnancy plans and fertility before treatment. Panitumumab requires contraception during treatment and for two months afterwards, and breastfeeding should be avoided for that period. When chemotherapy is also included, its precautions may last longer; ask for the instructions for the whole regimen.

Recognise changes early

You do not need to identify the responsible medicine before asking for help.

Track impact, not just a score

Describe when symptoms began, whether they are worsening, and whether they affect food, fluids, sleep, walking or daily tasks. Your team assigns the clinical grade. Use the passport if it helps; it is not required to access this information.

Visits, access and coordination

Know what happens in the clinic and what happens at home.

The delivery plan

Day 1: panitumumab infusion. There is no home chemotherapy pump in this regimen. A cycle lasts 14 days. Treatment continues while it is helping and side effects remain acceptable.

Radiotherapy or other treatment

This regimen has no routine radiotherapy component. If another treatment is proposed, ask whether it happens before, during or after this course and who will coordinate the appointments.

Questions worth asking

Why do I need a RAS test?

RAS is part of the growth-signalling pathway inside cancer cells. If it is changed by a mutation, blocking EGFR is unlikely to help. Your team needs a validated wild-type RAS result before panitumumab. Other tumour tests and the treatment aim also affect the choice.

Is this the same as checkpoint immunotherapy?

Panitumumab is a targeted antibody against EGFR. It works differently from checkpoint treatments used in some other cancer regimens. Ask your team which treatment class and side effects apply to you.

Does a rash mean the treatment is working?

A rash is a recognised drug effect, but it cannot tell you whether your cancer is responding. Report it early so it can be treated. Scans, symptoms and clinical review guide the response assessment.

Will I need radiotherapy as part of this cycle?

These panitumumab regimens do not contain a scheduled radiotherapy course. If radiotherapy is recommended separately, your oncology teams will explain its purpose and timing. Do not assume it occurs on infusion or pump days.

How long should I allow for the appointment?

The infusion itself is usually about an hour at first, but blood tests, review, pharmacy preparation and observation add time. Ask your unit for its expected visit length. Larger doses or a previous reaction may need a longer infusion.

Try explaining the plan back: Can you explain which medicines you are receiving, the schedule, the contact plan and the symptoms that need prompt help?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · antibody monotherapy

Active medicines: Panitumumab

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg PO once	Before panitumumab, according to the individual antiemetic prescription NCIS V8 low-risk row. Routine infusion-reaction premedication is a separate issue and is not usually required for panitumumab.
Metoclopramide	10 mg PO up to three times daily as required	Rescue nausea treatment only as prescribed NCIS V8 rescue option; check contraindications, renal function, interactions and prescribed duration.

Additional phase-specific support

- Antiemetic classification refers to the active treatment day. Reconcile duplicate steroid entries before prescribing.
- No automatic granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) schedule is added; use the patient's neutropenia history, risk and consultant plan.
- No routine cetuximab-style antihistamine/steroid premedication is imported into panitumumab.
- Provide local EGFR skin care, periodic magnesium/calcium/potassium checks and monthly post-treatment electrolytes for two months.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Skin / rash

Panitumumab

Report rash, cracks, painful or infected skin early.

2 Eyes

Panitumumab

New eye pain, light sensitivity or blurred vision: prompt advice.

3 Breathing

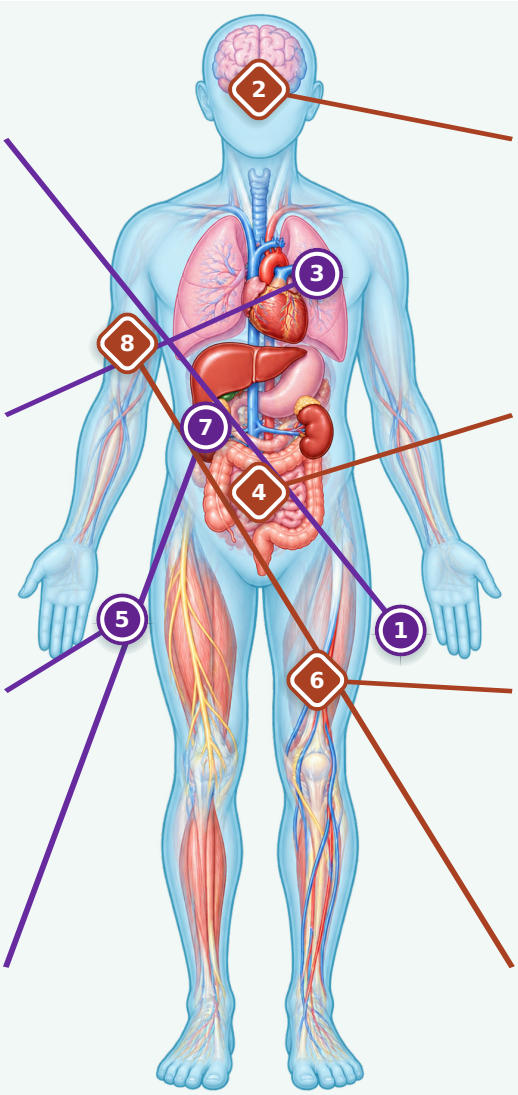
Panitumumab

New or worsening breathlessness or cough needs assessment.

4 Bowel

Panitumumab

Severe diarrhoea, dizziness or inability to drink: urgent contact.



5 Nails

Panitumumab

Painful or infected nail folds need early treatment.

6 Blood minerals

Panitumumab

Tests check magnesium, calcium and potassium; report cramps or weakness.

7 Hydration

Panitumumab

Reduced urine or severe dehydration needs urgent advice.

8 Infusion reaction

Panitumumab

Tell the nurse about swelling, itching, flushing or breathing changes.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report new breathing difficulty, eye pain or blurred vision, blistering or infected skin, severe diarrhoea or dehydration promptly.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Panitumumab · EGFR-targeted antibody

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Skin / nail changes	□ None	□ Mild change: report	□ Pain or daily tasks affected: call	□ Spreading infection or blistering: urgent review	□ Severe illness / collapse: emergency help
Eye symptoms	□ None	□ New irritation: report	□ Pain, redness or light sensitivity: call promptly	□ Blurred vision / severe eye pain: urgent review	□ Sudden major vision loss: emergency assessment
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Keep your treatment details to hand

Advanced / metastatic colorectal cancer · NCCP 00225 v6 · Document revised 5 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Panitumumab

Panitumumab Day 1 every 14 days.

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Rolling record; the team confirms the total treatment plan.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.34 · NCCP 00225 v6 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00225 v6 · official regimen**
   https://healthservice.hse.ie/documents/6560/225_Panitumumab_6mgkg.pdf
2. **Panitumumab product information ↗**
   https://www.medicines.org.uk/emc/product/6178/smpc
3. **Panitumumab patient medicine leaflet**
   https://www.medicines.org.uk/emc/product/6178/pil
4. **HSE: chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
5. **20100007 primary report**
   https://pubmed.ncbi.nlm.nih.gov/27736842/
6. **NCCP NCIS supportive antiemetic medicines V8 (2025)**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
7. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq